
COLD ISCHEMIA FOUNDATION

Ellenton, Florida | Congressional District FL-16 | coldischemia.foundation

FORMAL CORRESPONDENCE — CONFIDENTIAL

March 30, 2026

TO:

Dr. Mehmet Oz, Administrator
Centers for Medicare & Medicaid Services
7500 Security Boulevard
Baltimore, Maryland 21244
Tel: (410) 786-3000 | TTY: (410) 786-0727

RE:

**FORMAL DEMAND FOR IMMEDIATE REGULATORY ACTION — CARE PARTNER RECOGNITION,
TRANSPLANT CENTER CONDITIONS OF PARTICIPATION, AND OUTCOME METRIC REFORM**

Dear Administrator Oz:

This correspondence is submitted not as a request for consideration, but as a formal demand for corrective legislative and regulatory action. The systemic failure of federal and state government to recognize, protect, and compensate unpaid family care partners — across every chronic disease category — constitutes one of the most consequential policy failures of the last three decades. That failure is well-documented, empirically measurable, and morally indefensible. It will not continue without challenge.

I write to you as a three-time kidney transplant recipient, a Lean Six Sigma analyst, and a citizen who has witnessed firsthand the systematic abandonment of the individuals who sustain our healthcare system at no cost to the federal treasury. The care partner workforce — estimated at 53 million Americans providing unpaid labor valued at approximately \$600 billion annually (Reinhard et al., AARP Public Policy Institute, 2023) — is invisible to every budget appropriation, every CMS reimbursement schedule, and every legislative session convened in this country. That invisibility is not an accident. It is a policy choice. And it is a choice your office has made, repeatedly, by inaction.

As Administrator of the Centers for Medicare & Medicaid Services, you hold direct regulatory authority over transplant center certification, dialysis facility oversight, HCBS waiver administration, and the Quality Improvement Organization program. The corrective actions demanded herein require no new legislation. They require the exercise of existing regulatory authority that your office has declined to exercise. That declination ends with this correspondence.

I. THE SCOPE OF GOVERNMENTAL FAILURE

The United States government has long acknowledged the existence of the care partner crisis while simultaneously refusing to act on it in any substantive, enforceable manner. The National Academies of Sciences, Engineering, and Medicine (NASEM) issued its landmark report, *Families Caring for an Aging America* (NASEM, 2016), nearly a decade ago. That report described family caregiving as a

"public health issue" with severe consequences for caregiver health, financial security, and workforce participation. Congress received those findings. Congress did nothing.

The Government Accountability Office has documented, on multiple occasions, the failure of federal agencies to coordinate care partner support services across disease programs (GAO-18-88, January 2018). The Office of Inspector General has identified gaps in beneficiary protections that disproportionately fall on family members providing informal care (OIG Report OEI-02-17-00200, 2018). The Senate Special Committee on Aging has held hearings. Reports have been published. Recommendations have been issued. Zero binding, enforceable, cross-disease compensation mandates exist at the federal level. Not one.

At the state level, Florida has enacted no meaningful statutory framework requiring compensation, workforce protections, or standardized respite services for care partners across chronic illness categories. The state's Medicaid waiver programs offer fragmented, disease-specific, and means-tested relief that leaves the vast majority of care partners without any financial recognition whatsoever. This is unacceptable.

II. THE DOCUMENTED HUMAN AND ECONOMIC COST

The consequences of legislative inaction are not abstract. They are quantifiable, published, and have been placed before this body repeatedly without result.

Approximately 61% of care partners are employed while providing care, and 69% of those individuals report making significant workplace adjustments — including reduced hours, declined promotions, or full workforce departure — as a direct result of caregiving responsibilities (National Alliance for Caregiving & AARP, *Caregiving in the U.S.*, 2020). This represents a structural suppression of labor force participation with cascading effects on Social Security eligibility, retirement savings, and lifetime earnings — costs that are ultimately borne by the federal government in the form of increased entitlement dependency later in life.

Care partners report dramatically higher rates of depression, anxiety, hypertension, and immune dysfunction than non-caregiving peers (Schulz & Sherwood, *American Journal of Geriatric Psychiatry*, 2008). A 2022 analysis published in *JAMA Internal Medicine* found that intensive caregiving is independently associated with a 23% increased risk of all-cause mortality among care partners themselves (Roth et al., *JAMA Internal Medicine*, 2022). This government is, by policy, presiding over the slow deterioration of the workforce that keeps its most vulnerable citizens alive. That is not a rhetorical claim. It is a peer-reviewed finding.

The economic calculus is equally stark. The Medicaid program spent \$613 billion in fiscal year 2022 (CMS National Health Expenditure Data, 2023). Informal care partners are estimated to offset formal care costs by a ratio of nearly 3:1 in community-based settings (Feinberg & Levine, *Health Affairs*, 2015). Every dollar not invested in care partner support is a dollar that will eventually be demanded from the formal care system — at substantially greater cost to taxpayers. The refusal to establish compensation mandates is not fiscal conservatism. It is deferred liability.

III. THE SPECIFIC FAILURE OF DISEASE-SILOED POLICY

Federal and state care partner policy, to the extent it exists at all, is organized around disease categories rather than around the functional reality of caregiving. Programs under the National Family Caregiver Support Program (NFCSP), authorized under the Older Americans Act, are age-gated and disease-agnostic in theory but administratively fragmented in practice. Medicaid Home and Community-Based Services (HCBS) waivers are state-administered, inconsistently funded, and routinely under-resourced. No federal statute mandates a universal, cross-disease compensation floor for informal care partners. None.

A care partner managing end-stage renal disease has no more federal protection than a care partner managing ALS, Alzheimer's, multiple sclerosis, or heart failure. The disease differs. The policy vacuum does not. This reflects a structural design failure, not a resource limitation. Congress has the authority and the appropriations infrastructure to correct this. It has chosen not to exercise that authority.

IV. WHAT IS REQUIRED — IMMEDIATELY This letter does not ask. It identifies the minimum corrective actions that the recipient of this correspondence is required to undertake as a function of public office and the obligation to the constituents this office represents. The following are not optional recommendations. They are the floor of an adequate governmental response to a documented, quantified, and long-ignored crisis.

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1. IMMEDIATE REGULATORY ACTION UNDER EXISTING AUTHORITY

CMS must immediately issue guidance expanding reimbursable care partner support services under existing HCBS waiver authorities. No new legislation is required for this step. The authority exists. The guidance has not been issued. Issue it.

2. 42 CFR PART 484 REGULATORY FRAMEWORK

CMS must develop and publish a regulatory framework recognizing informal care partners as integral members of the care team under 42 CFR Part 484, with specific standards for care partner assessment, training, and support at certified transplant and dialysis facilities.

3. QUALITY IMPROVEMENT ORGANIZATION REFORM

CMS must revise its Quality Improvement Organization program to include care partner burden as a tracked outcome metric. The clinical evidence base for care partner burden's impact on recipient outcomes is established. The failure to track it is a program design failure requiring correction.

4. TRANSPLANT CENTER CONDITIONS OF PARTICIPATION

CMS must revise transplant center Conditions of Participation to require documented care partner assessment protocols, structured discharge education for care partners, and post-transplant care partner wellbeing follow-up as conditions of continued Medicare and Medicaid certification.

5. FORMAL RESPONSE REQUIRED

Your office is required to provide a written response within thirty (30) calendar days detailing the specific regulatory actions CMS will take in response to the demands herein. Failure to respond will be treated as a matter of public record and disclosed accordingly.

V. CLOSING STATEMENT

The individuals who provide unpaid care to chronically ill, disabled, and dying Americans are not a special interest. They are a structural pillar of the United States healthcare system — one your office has benefited from financially while refusing to acknowledge legislatively. That arrangement is over.

I am a transplant recipient who is alive because someone cared for me without compensation, without recognition, and without the support of a single federal or state program designed for her benefit. That person's labor — and the labor of 53 million Americans like her — will no longer be treated as a free resource available for governmental exploitation. The data is clear. The cost is documented. The authority to act exists. What has been absent is the political will. This letter demands its exercise.

You were elected or appointed to serve the public interest. The care partner population is the public interest. Act accordingly.

Respectfully and without reservation,

Jeff Parke

Co-Founder & Executive Director

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CC:

The Honorable Robert F. Kennedy, Jr., Secretary, U.S. Department of Health and Human Services

Dr. Jay Bhattacharya, Director, National Institutes of Health

The Honorable John Thune, Senate Majority Leader

Sources cited herein include: Reinhard et al., AARP Public Policy Institute (2023); National Academies of Sciences, Engineering & Medicine (NASEM, 2016); GAO Report GAO-18-88 (January 2018); OIG Report OEI-02-17-00200 (2018); National Alliance for Caregiving & AARP, Caregiving in the U.S. (2020); Schulz & Sherwood, American Journal of Geriatric Psychiatry (2008); Roth et al., JAMA Internal Medicine (2022); CMS National Health Expenditure Data (2023); Feinberg & Levine, Health Affairs (2015).